Wallace Pharma Walyte Solution

1. Brand and objectives

Focus brand

- Walyte – Oral Rehydration Salts (ORS), Wallace Pharma

Current status

- No. 2 brand in the ORS category.
- Roughly **one-fourth the size of the market leader**.

Key challenges

1. **WHO ORS vs "electrolyte drinks" confusion**
   - Doctors understand WHO-recommended ORS formulations.
   - Parents and retailers often do not – they treat all "salts / electrolyte drinks / energy drinks" as similar.
   - Incorrect choices are common, especially in pharmacies and at home.
2. **Large gap vs the category leader**
   - Leader is 4–5x larger.
   - They have already started social-media led education on ORS.
3. **Need to broaden ORS use beyond diarrhoea, within safe limits**
   - Historically ORS is seen only as "for diarrhoea".
   - Medical teams want to emphasise:
     - ORS as the **first line for dehydration from diarrhoea and vomiting**,
     - And its role in **mild dehydration from heat, fever, and exertion**, as guided by paediatric experts.

4. **Declining diarrhoea incidence → shrinking core market**
    - Better water access and hygiene have reduced diarrhoeal illness.
    - Fewer diarrhoea cases = fewer “automatic” ORS prescriptions.
5. **Previous doctor education exists; now need parent education**
    - A mini-CME program was done with a different division.
    - This division now wants to **educate parents directly** so they:
        - Recognise dehydration risk early, and
        - Choose WHO-recommended ORS instead of random “electrolyte” products.

Brand objectives

1. Build a **clinic-intermediated dehydration safety service** that parents use at the first sign of risk.
2. Make WHO-recommended ORS the **default choice** in parents’ minds for dehydration, not juices / soft drinks / energy drinks.
3. Position Walyte as the **natural brand choice** when doctors reach for WHO ORS – by being the brand that funded and distributed this clinic service.

All clinical education content is **academy-branded, molecule-only (“ORS / WHO- recommended ORS”)**.

Walyte appears only on the **last-page advertisement** of doctor PDFs.

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2. Solution approach and rationale

Solution approach

Create an **IAP-aligned “Dehydration & ORS Safety Service”** for clinics, built on:

1. A **clinic-branded microsite** with a short **dehydration screening form** and a local-language **explainer video** on the output page.
2. A simple **clinic poster + QR code**, with dehydration signs, guiding parents to use the service whenever they suspect dehydration.
3. A **short IAP micro-CME PDF series for paediatricians**, reinforcing when and how to use ORS beyond classic diarrhoea, and how to explain WHO

ORS to parents.

- Each CME PDF is academy-branded, molecule-level.
- The last page is reserved as a **Waylte advertisement page**.

Rationale

- Parents **often miss dehydration risk** until it is severe, or they treat it with the wrong drinks.
- Doctors spend time repeating the same “ORS vs juices / sodas / ‘energy drinks’” counselling in every visit.
- Chemists sometimes push non-ORS products, because parents do not know what to ask for.

A **clinic-branded, digital dehydration tool**:

- Catches parents **at the moment of risk** (loose motions, vomiting, fever, heat, sports).
- Guides them towards:
  - Early doctor visit when needed.
  - Correct use of **WHO-recommended ORS** when home management is safe.
- Builds **clinic loyalty**, because the service carries the clinic’s identity and language.

Waylte, as the sponsor of the Clinic-Patient-Education service becomes the **default WHO ORS in the doctor’s mind** when they connect this clinic service to a pack on the shelf.

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3. Solution Components

A. Clinic-branded “Dehydration & ORS Safety” microsite

What it is

- A simple, mobile-friendly local language **microsite**, **white-labelled for each clinic**:
- Content approved by IAP or SAPA.
- Accessible via **QR code and short link**.

How parents reach it

- A **clinic poster / standee** in the waiting area and near the reception, saying for example:

  "If your child has loose motions, vomiting, high fever, or has been in strong heat or sports for many hours, please scan this code to check for dehydration and see what to do next."

- Parent scans QR or taps the link they receive from clinic staff (WhatsApp).

Screening form (on the microsite)

Short, local-language questions:

- Child's age band
- Current problems (multi-select):
  - Loose motions
  - Vomiting
  - Fever
  - Playing or working in heat / sun
  - Long sports / intense exercise
- Has the child:
  - Been drinking normally?
  - Passed urine in the last 6–8 hours?
  - Tears when crying?
  - Been active / playful, or more dull / sleepy?
- Any blood in stool or vomit?
- Any severe pain, breathing difficulty or confusion?

Output page + video

Based on the answers, the microsite shows a **clinic-branded output page** that:

- Groups the child into a simple zone:
  - **Green (mild risk):**
    - "You can start treatment at home now using WHO-recommended ORS."

        - Simple text explaining:
            - How to prepare ORS correctly.
            - How often to give (small sips, after each loose stool / vomit, more during heat/exertion as guided by the academy).
            - When to re-check or repeat the form.
    - **Amber (needs doctor visit soon):**
        - “Your child should see the doctor today.”
        - Lists concerning signs and why a clinic visit is needed.
    - **Red (emergency):**
        - “Take your child to the nearest hospital or emergency room now.”
        - Lists severe danger signs; asks parent to inform staff that the child is a patient of this clinic.
- On the same page, an embedded **local-language explainer video** plays:
    - “How to recognise dehydration in children and when to use WHO-recommended ORS.”
    - Explains, in very simple terms:
        - What dehydration looks like.
        - Why ORS is different from water, juices and energy drinks.
        - When ORS can be started at home and when a doctor or hospital is needed.
    - The same core video is produced in **major Indian languages**; the microsite automatically serves the selected language.

There is **no pharma logo, no brand name** on the microsite, form, output or video.

B. Micro-CME PDFs for pediatricians

To support the clinic microsite and shape the doctor’s default ORS brand choice, we add a small IAP-licensed **micro-CME PDF series**.

Format

- 3 short PDFs, spaced over 3 months.

- Each PDF:
  - 2–3 pages of academy-branded content on ORS and dehydration.
  - Optionally a small 3–5 question quiz.
  - A final **advertisement page reserved for Walyte**.
- Shared via **dIAP / digital tool and WhatsApp**, and optionally printed.

Proposed topics

1. **Module 1 – "Dehydration in children – not just diarrhoea"**
   - Practical overview of dehydration risk in:
     - Diarrhoea and vomiting,
     - Fever with poor intake,
     - Summer heat, sports and exertion.
   - When ORS at home is appropriate, and when clinic / hospital is needed.
2. **Module 2 – "Why WHO-recommended ORS is different from other drinks"**
   - Simple explanation of composition and osmolarity at a clinic-communication level.
   - Why juices, soft drinks, homemade salt-sugar guesses and "energy drinks" **do not replace ORS**.
   - How to guide parents to read "WHO-recommended ORS" on packs.
3. **Module 3 – "One-minute script: explaining dehydration and ORS to parents"**
   - Ready-to-use scripts for explaining:
     - What dehydration is.
     - When ORS is enough.
     - When to come back / go to hospital.
   - How to point parents to the **clinic microsite & video** instead of repeating long explanations.

Walyte placement

- Each module is IAP content on the medical pages.
- The last page is a **Walyte advertisement page**, which can:

- Highlight “Walyte = WHO-recommended low-osmolarity ORS”.
- Show available flavours / forms for children.
- Connect to the dehydration microsite message (e.g., “matches what you tell parents in your clinic service”).

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4. Expected outcomes

For pediatricians and clinics

- An **easy, consistent way** to handle dehydration counselling:
  - Instead of repeating the same talk every time, they can ask parents to use the microsite and watch the video.
- Parents arrive more informed:
  - Recognise dehydration signs better.
  - Understand why ORS is needed and what “WHO-recommended ORS” means.
- Doctors can extend ORS teaching beyond diarrhoea, within safe and guideline-aligned boundaries (fever, heat, sports), without feeling they are “selling” a product.
- The clinic’s identity is strengthened by a **clinic-branded safety service**.

For parents and children

- **Earlier action** on dehydration risk:
  - Parents are nudged to check risk as soon as symptoms appear.
- **Correct first step** at home:
  - Use WHO-recommended ORS instead of juices / cola / random “electrolytes”.
- Better understanding of:
  - When to manage at home,
  - When to see the doctor,
  - When to go to hospital.

- Clear, local-language video guidance reinforces what the doctor says, reducing confusion and anxiety.

For Walyte

- Walyte becomes the brand behind the **Dehydration & ORS Safety Service** that clinics value.
- In doctors' minds, the phrase "WHO-recommended ORS" is tied to the clinic service, re-inforced by the three in-clinic education pieces.
- Helps close the gap with the market leader by:
  - Increasing **category use** in non-diarrhoeal dehydration situations, and
  - Improving **brand choice** when a doctor or chemist must pick a WHO ORS.
- Provides a clear, education-led story for internal teams and leadership.

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5. Commercials

One-time development

1. **Microsite and video**
   - Script and content development for:
     - Dehydration screening questions and logic (green/amber/red outputs).
     - Local-language explainer video ("Recognising dehydration and using ORS correctly").
   - Video production in **major Indian languages**.
   - Design and build of the **clinic white-label microsite**.
   - Design of clinic posters / standees and QR / link assets.
2. **Micro-CME PDFs**
   - Medical content development for 3 academy-branded modules.
   - Layout and design of PDFs.
   - Integration into **dIAP / sharing tool**.

Ongoing / annual

- **Microsite hosting and maintenance**, including language and content updates.
- **Distribution support** via sharing tool and simple field-usage dashboards.

Suggested commercial model:

- **₹1,500 per clinic per year** for activation of the Dehydration & ORS Safety microsite (including poster, QR, hosting, and basic usage reporting).
- **₹45 per clinic visit** where a micro-CME PDF is shared with the doctor.
- Minimum of **3,000 clinics** for the first year, with scope to scale up.

Reporting

- Periodic (e.g., monthly) reports on:
  - Number of clinics onboarded and active.
  - Number of screening form completions and video views (aggregate).
  - Number of micro-CME PDFs shared / opened, and reads up to the last page.

This makes Waylte the brand that **owns "dehydration & ORS safety" around the clinic**, while staying fully aligned with academy guidance and keeping all patient-facing content clinic-branded and brand-neutral.